



Report, Oct. 2024-Feb 2025 | Health Matters Clinic

Understanding Healthcare Access and Experiences in Skid Row, Los Angeles

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Important Terms:

Street Medicine is a healthcare approach that delivers medical services directly to individuals experiencing homelessness, typically on the streets, aiming to provide care where people live rather than in traditional healthcare facilities.

Mobile Health refers to healthcare services provided through mobile health units—vehicles equipped with medical equipment and staffed by healthcare professionals—that travel to various locations to offer care, screenings, and treatment, often in underserved areas.

OVERVIEW

Skid Row, Los Angeles is one of the most densely populated areas for people experiencing homelessness in the United States. With thousands lacking stable housing, health conditions become exacerbated due to poor living conditions, lack of sanitation, and limited access to healthcare services. This includes the spread of communicable diseases such as tuberculosis, hepatitis, HIV, and COVID-19, as well as the aggravation of chronic conditions such as diabetes, hypertension, substance use disorders, and mental health disorders.

Barriers to accessing healthcare in Skid Row include lack of identification, unstable living situations, transportation issues, and mistrust of the healthcare system. Several healthcare providers operate in Skid Row that offer services for free or at a low-cost, but they are often overburdened due to high demand, making it difficult for individuals to even get an appointment. Medi-Cal, California's Medicaid program, is available to many low-income and unhoused individuals, but survey results highlight that various social determinants of health, such as access to transportation and medical mistrust, create barriers to receiving healthcare in traditional settings despite being insured.

This survey seeks to gain the direct perspectives of individuals residing in Skid Row, Los Angeles regarding their healthcare access and experiences, as well as understand the types of services that individuals would find most helpful. Key findings include:

- **85.5% of respondents said that they found street medicine services helpful, with an additional 13.0% saying that they were sometimes helpful.**
- **52.0% of individuals surveyed reported that accessing medical services is challenging, with the main reasons being transportation difficulties and challenges with insurance.**
- **87.1% of respondents say they are somewhat likely or very likely to use street medicine/mobile health services again.**
- **The most sought after service in Skid Row is food assistance (71.2%), followed by housing assistance (65.8%), HIV testing+counseling (47.9%), primary care (45.2%), mental health care (45.2%), COVID testing (45.2%), and education services (42.5%).**
- **21.3% of respondents shared street medicine/mobile health teams would be more helpful if they improved continuity of care.**
- **42.4% of those who had received services from street medicine/mobile health teams had only ever received those specific services on the streets.**
- **48.0% of those who reported lack of trust in the doctor cited dismissal of symptoms and perceived discrimination/stereotyping.**

Efforts to enhance healthcare access in Skid Row continue to be notable, yet the demand greatly surpasses available resources, and **significant gaps remain in meeting basic health needs even with current street medicine/mobile health models in place.**

This report seeks to provide street medicine leaders and policymakers with insights into the healthcare challenges faced by Skid Row residents, using their direct voices and perspectives to better **shape policies aimed at improving healthcare for unhoused populations in Los Angeles.**

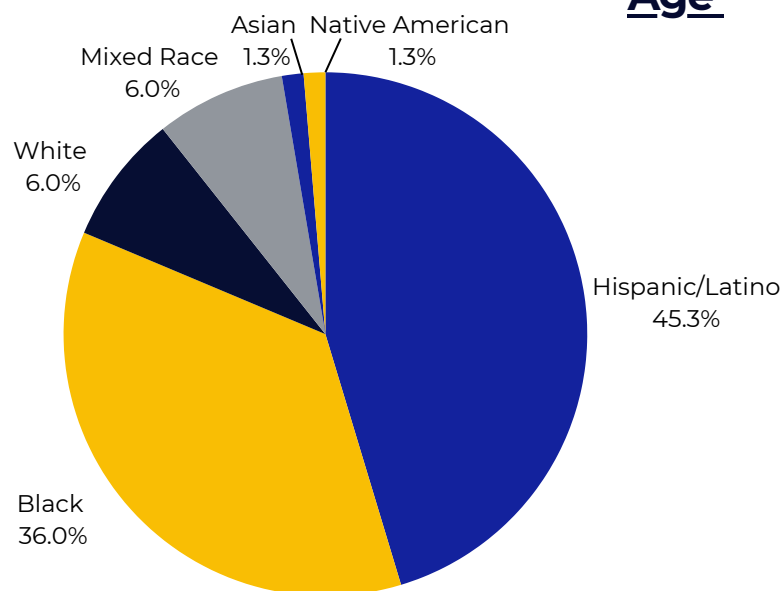
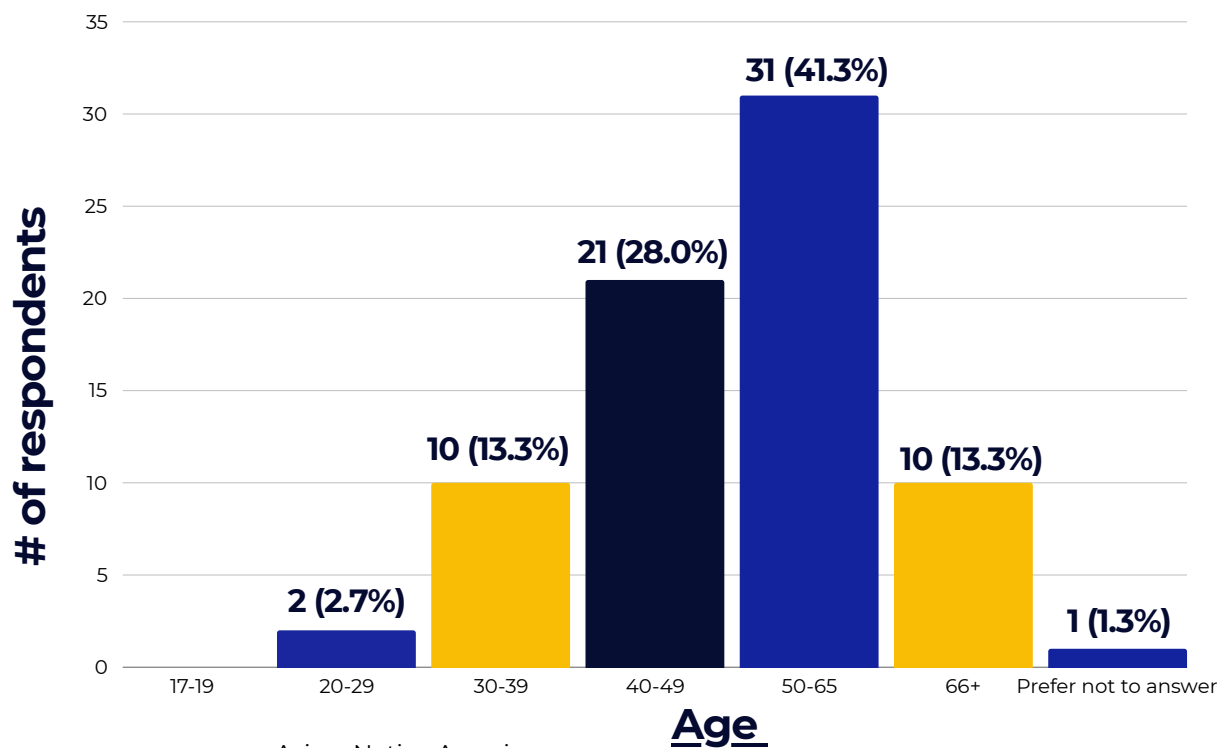
METHODOLOGY

Participants for this survey were approached throughout Skid Row and asked if they would be interested in completing a survey to better understand their healthcare access and experiences. Participants gave their consent to participate orally. The inclusion criteria required that respondents be adults over the age of 18, capable of providing informed consent, fluent in either English or Spanish, and either unhoused or living within a housing facility in Skid Row.

Participants were informed that their involvement in the survey was entirely voluntary, with the option to skip any question or discontinue participation at any time. No compensation or incentives, such as gift cards or monetary rewards, were provided for participation. No personal identifying information was collected. Surveys were conducted in an interview-based format to accommodate individuals who may not be able to read, write, or navigate technology. Responses were recorded during the interviews and subsequently analyzed for themes using Survey Monkey.

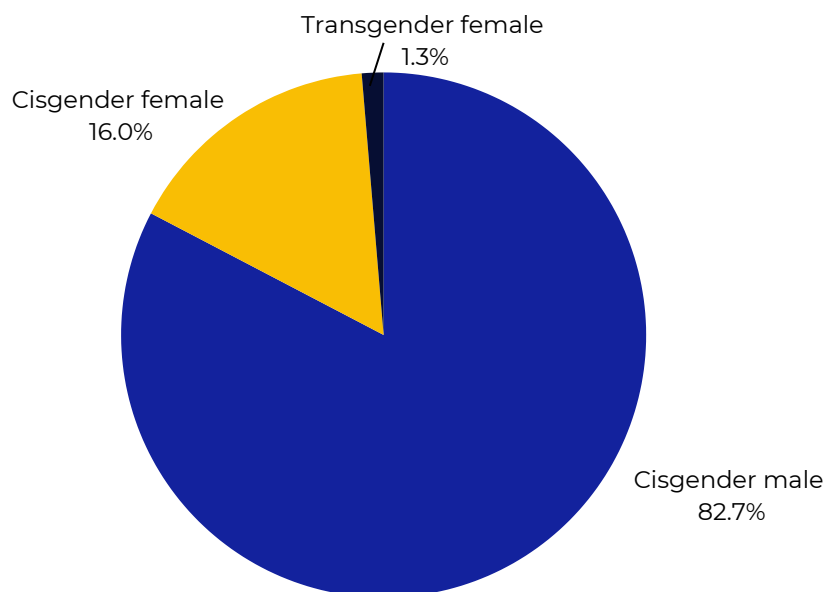
This study has been certified as exempt from IRB review under 45 CFR 46.104, category 2, by the UCLA IRB, determined on October 3, 2024. Data was collected from **75 participants between October 2024 and February 2025.**

DEMOGRAPHICS



Race/Ethnicity:

34 (45.3%) Hispanic/Latino
 27 (36.0%) Black
 6 (8.0%) White
 6 (8.0%) Mixed Race
 1 (1.3%) Native American
 1 (1.3%) Asian



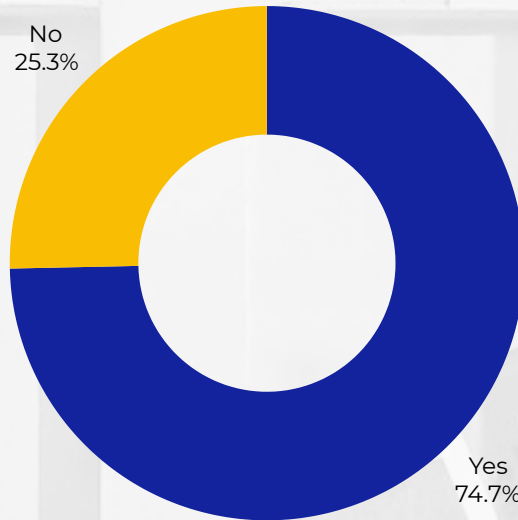
Gender Identity:

82.7% (62) Cisgender male
 16.0% (12) Cisgender female
 1.3% (1) Transgender female

"Are you currently experiencing homelessness?"

Yes: 74.7% (56)

No: 25.3% (19)



"How long have you been experiencing homelessness?"

Less than 1 year: 21.4 % (9)

1-3 years: 35.7% (15)

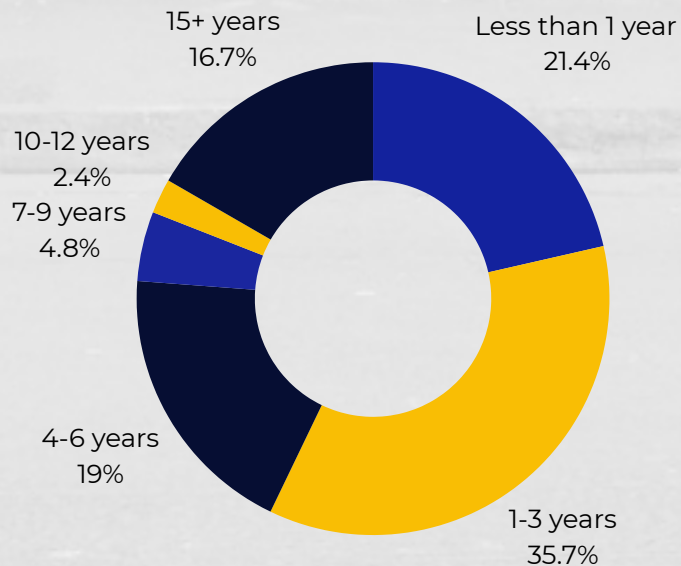
4-6 years: 19.0% (8)

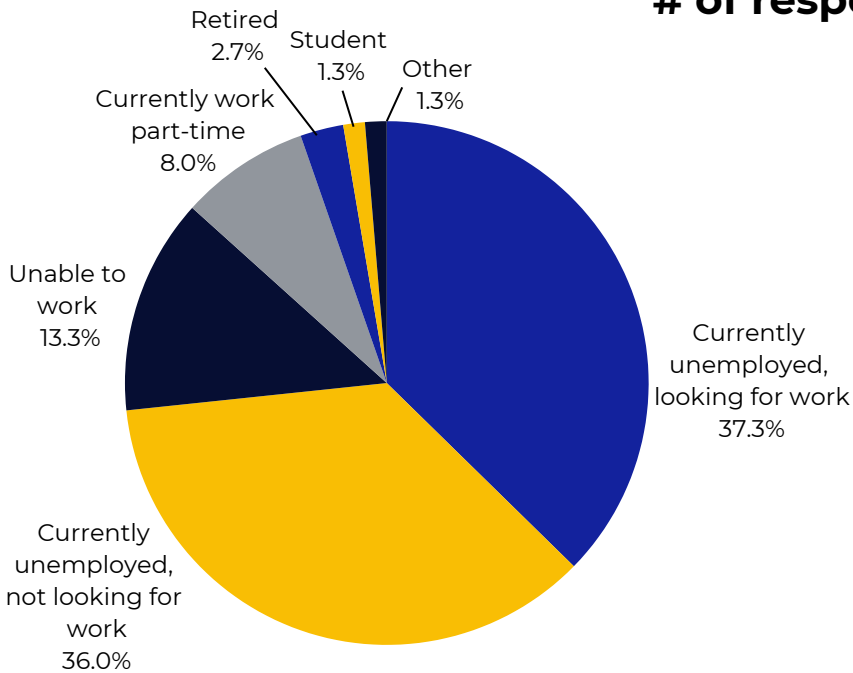
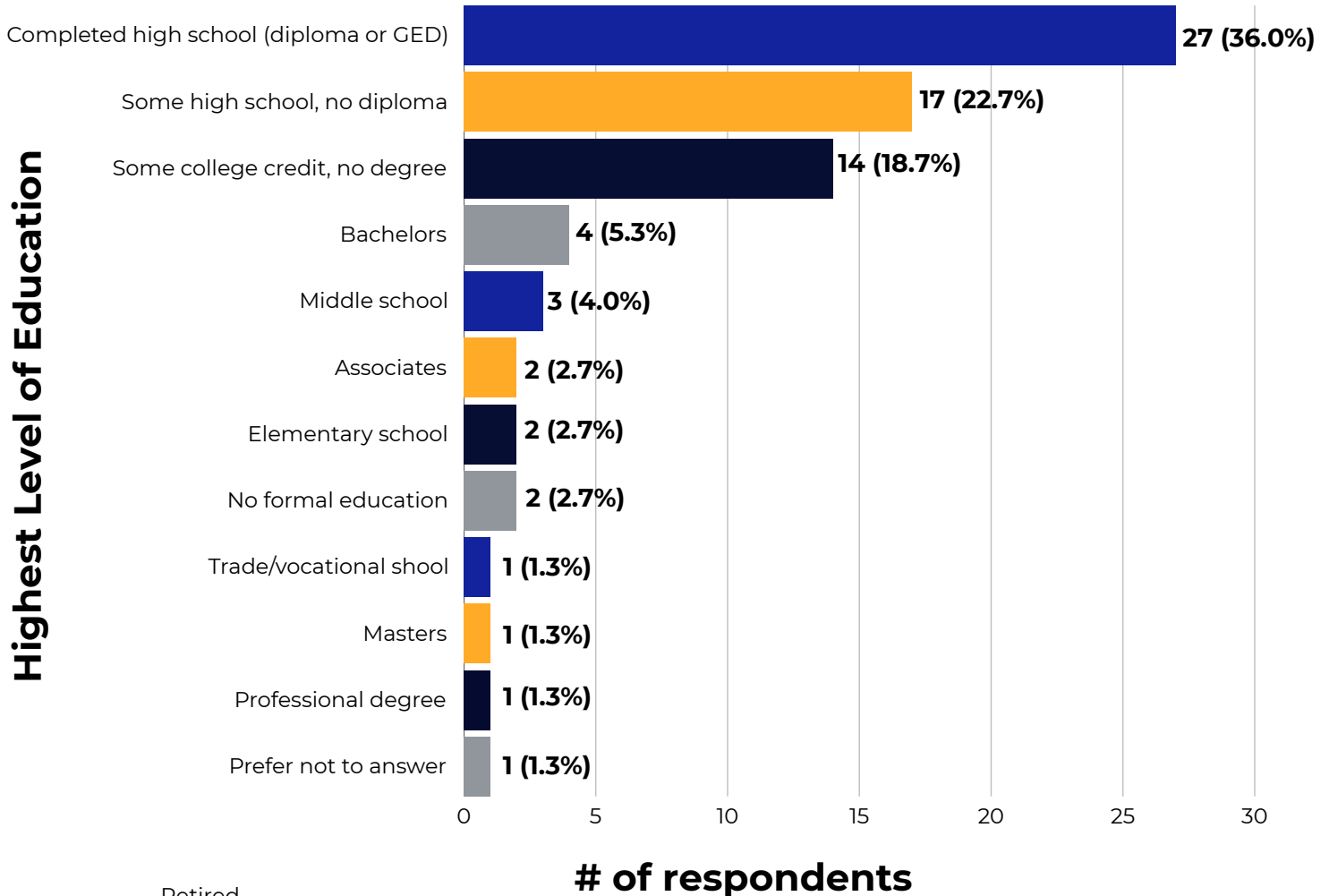
7-9 years: 4.8% (2)

10-12 years: 2.4% (1)

13-15 years: 0% (0)

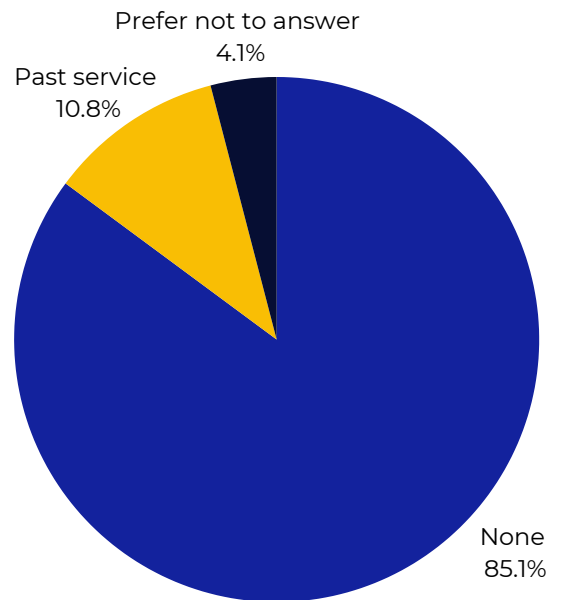
15+ years: 16.7% (7)





Employment Status:

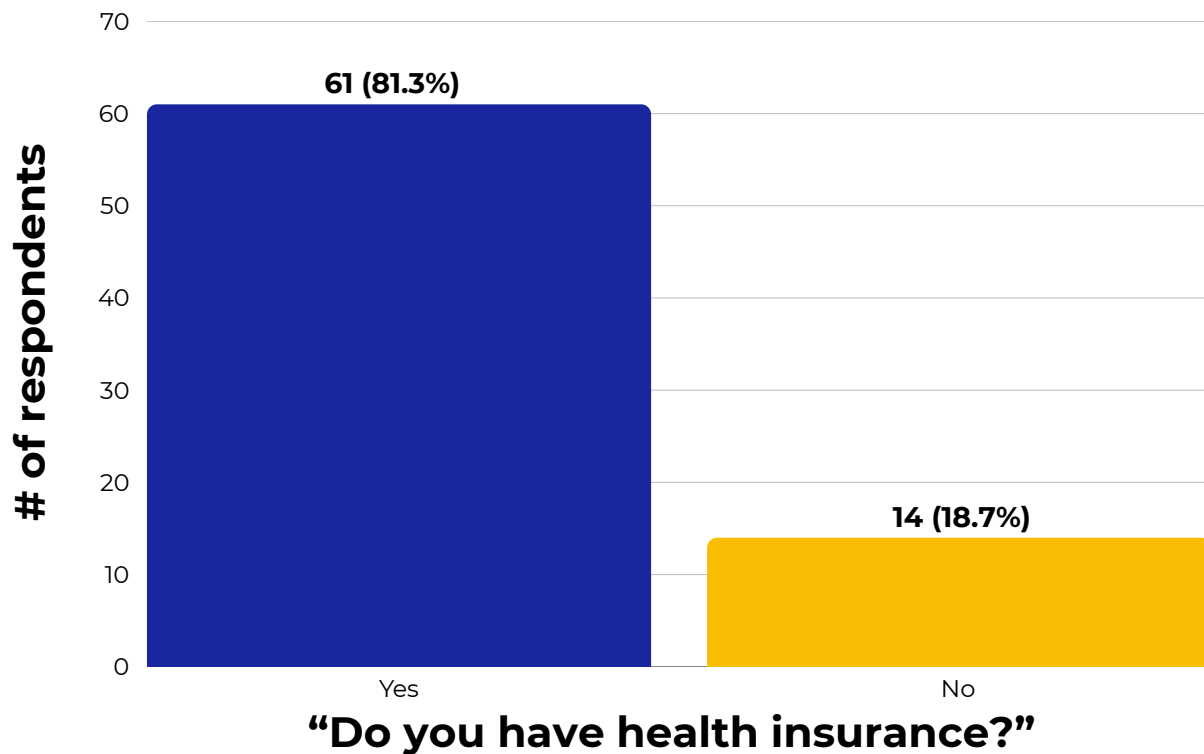
28 (37.3%) Currently unemployed, looking for work
 27 (36.0%) Currently unemployed, not looking for work
 10 (13.3%) Unable to work
 6 (8.0%) Currently work part-time
 2 (2.7%) Retired
 1 (1.3%) Student
 1 (1.3%) Other



Military Status:

63 (85.1%) None
 8 (10.8%) Past service
 3 (4.1%) Prefer not to answer

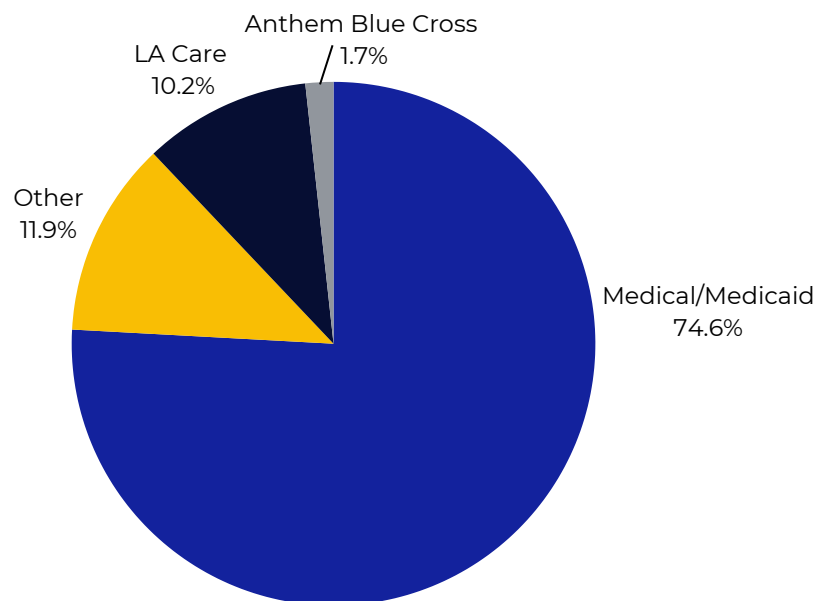
HEALTHCARE ACCESS



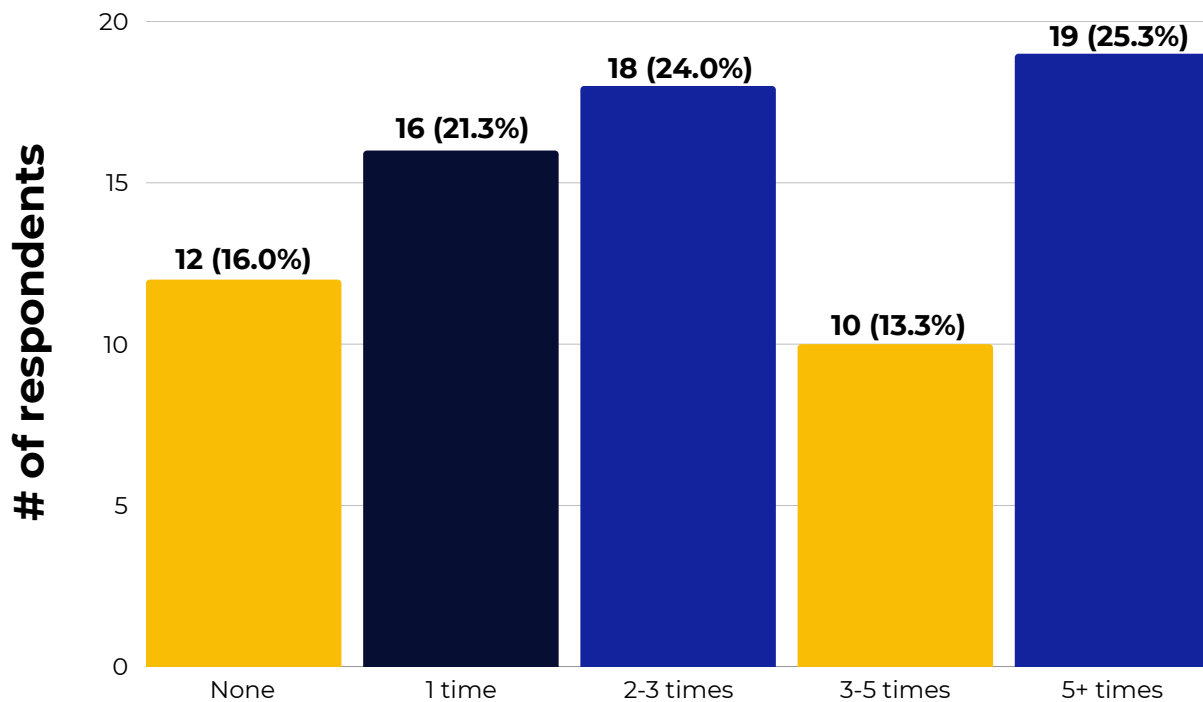
“Who are you insured by?”:

Medical/Medicaid: 44 (74.6%)
Other*: 7 (11.9%)
LA Care: 6 (10.2%)
Anthem Blue Cross: 1 (1.7%)

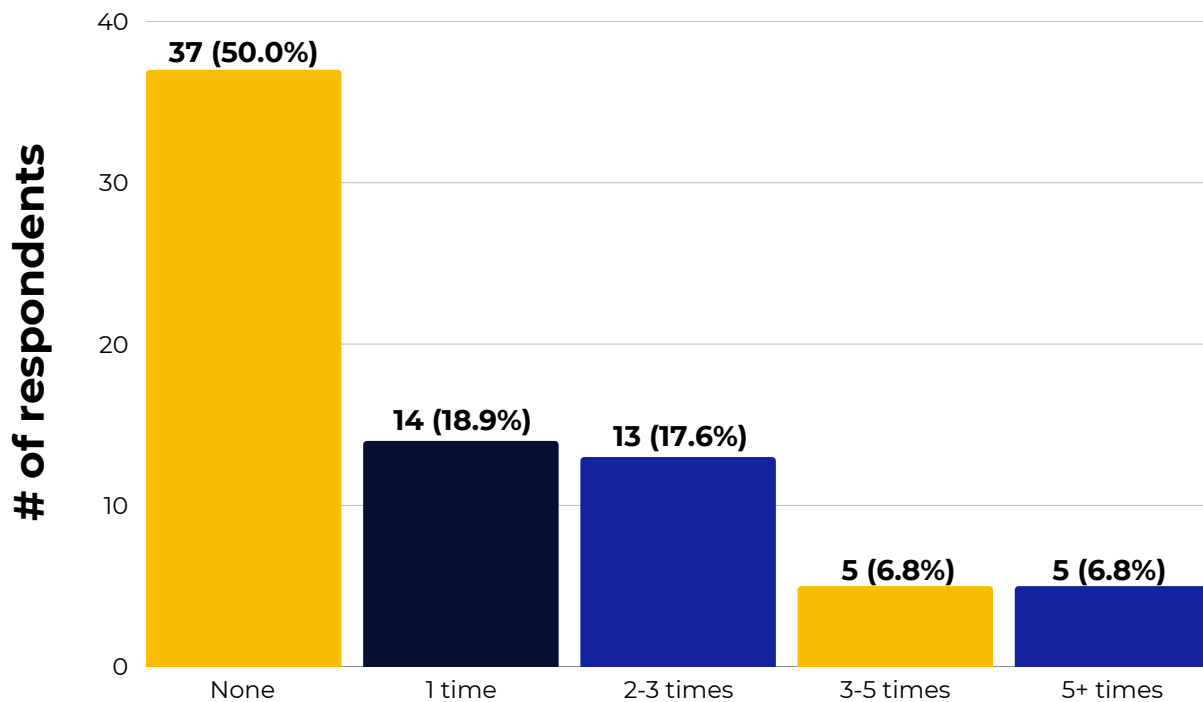
**Other responses include HealthNet, CalPERS, Blue Cross Blue Shield, and Molina*



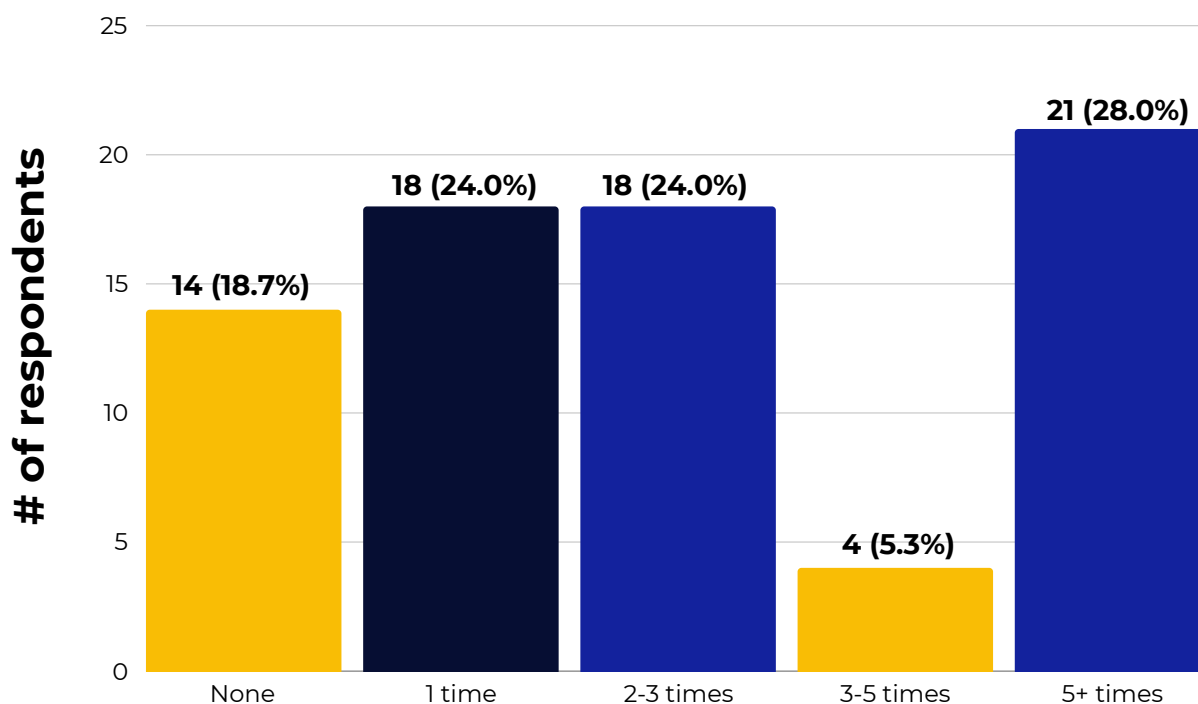
“How many times per year do you receive care in a doctor’s office?”



“How many times per year do you receive care in the emergency room?”



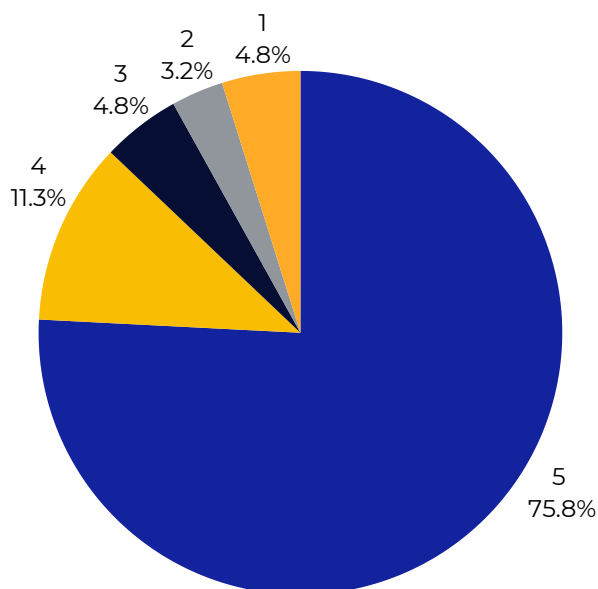
“How many times per year do you receive care from street medicine/mobile health teams?”



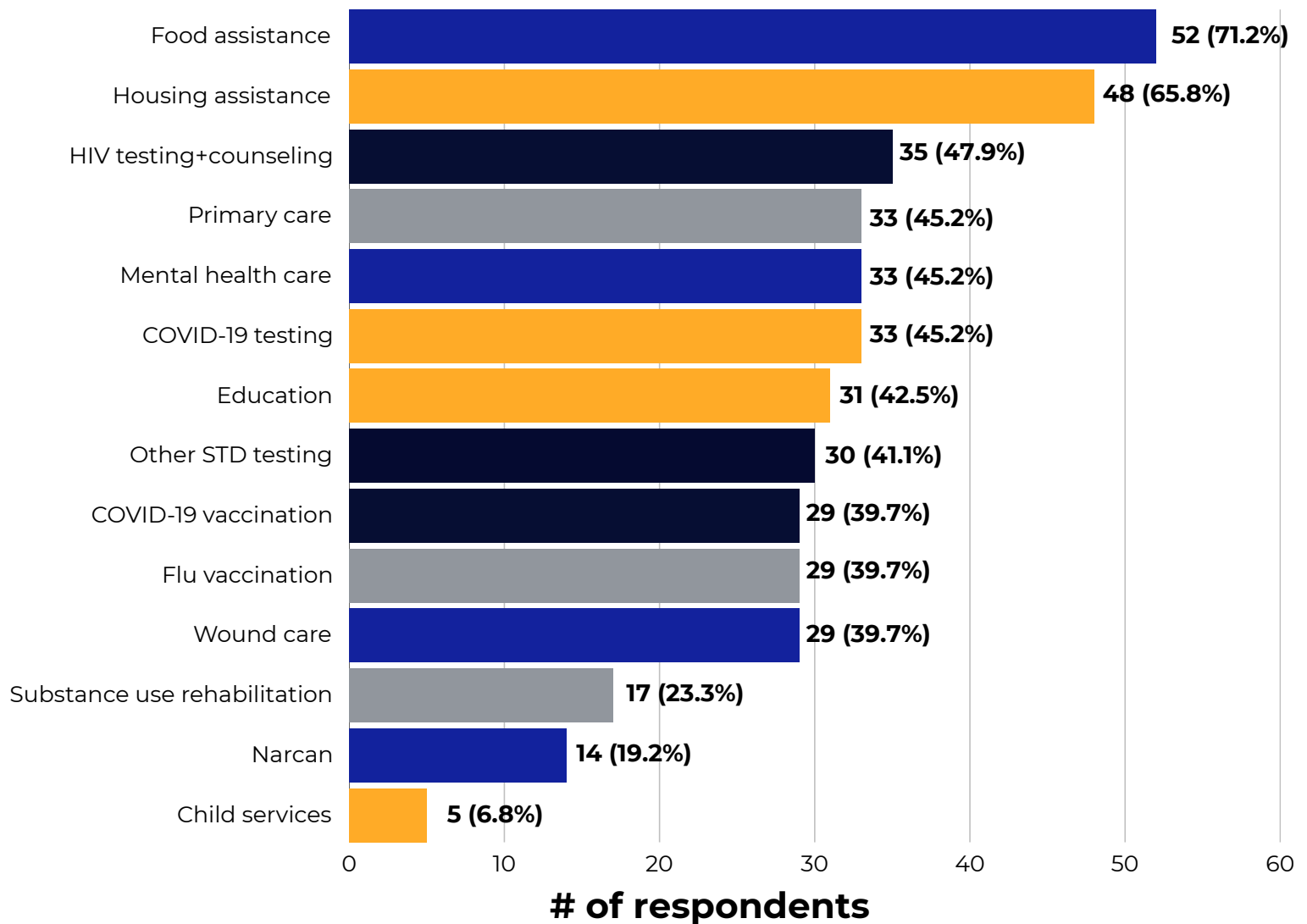
Among those who reported using mobile health/street medicine services more than once a year (62 respondents), 87.1% reported being likely to use these services again, with 75.8% saying “very likely” and 11.2% saying “somewhat likely.” **It is evident that mobile health/street medicine services fulfill a critical need within the Skid Row population.** This strong inclination toward future utilization suggests that mobile health services are not only providing immediate medical assistance but also fostering trust and accessibility among users, which is vital for improving health outcomes.

“If you have received care from mobile health/street medicine services, how likely are you to use them again?” (on a scale from 1-5)

- 5 (very likely): 47 (75.8%)
- 4 (somewhat likely): 7 (11.3%)
- 3 (neutral): 3 (4.8%)
- 2 (not likely): 2 (3.2%)
- 1 (never): 3 (4.8%)



REQUESTED SERVICES



The most frequently reported services that respondents requested needing access to were food assistance services (71.2%), housing assistance (65.8%), HIV testing + counseling (47.9%), primary care (45.2%), mental health care (45.2%), COVID-19 testing (45.2%), and education services (42.5%). These findings highlight the need for comprehensive medical and social support that address the multifaceted challenges faced by the Skid Row population.

"Other services" requested included legal services, carpool services, blood sugar checks, footwear donations, assistance filing for documentation, and self-defense lessons.

Note that respondents were allowed to select multiple options

HEALTHCARE EXPERIENCES

- Despite over 80% of respondents reporting some form of health insurance, **a disconnect persists between coverage and actual care received by respondents in Skid Row.**
- Alarming, ~20% of respondents reported not visiting a doctor at all in any given year (whether in a traditional doctor's office, emergency room, or mobile health service), particularly those experiencing homelessness for 10 years or longer.
- This, coupled with nearly half of the population reporting unmet needs for crucial services like food assistance services (71.2%), housing assistance (65.8%), HIV testing + counseling (47.9%), primary care (45.2%), mental health care (45.2%), COVID-19 testing (45.2%), and education services (42.5%) **suggests that insurance alone is not enough to bridge the gap in healthcare access.**

The **subsequent interview-based questions** are designed to explore in more detail the barriers in access to healthcare in traditional settings (ex: the doctor's office) that respondents face, as well as their experiences in traditional healthcare settings and street medicine/mobile health services.

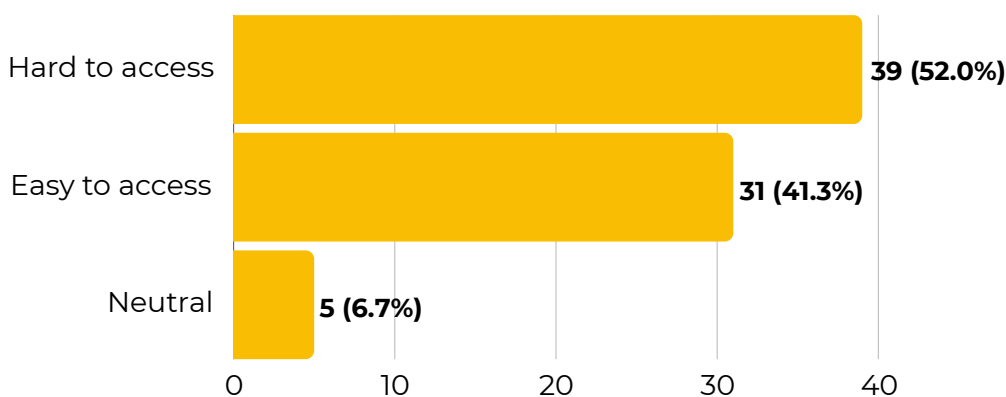


“How do you feel about visiting the doctor's office?”

While 45.3% of respondents to this question (72 respondents) reported generally positive feelings about visiting the doctor's office—most commonly stating "I feel comfortable"—**a notable portion expressed less favorable sentiments**. Specifically, 33.3% of respondents shared negative emotions about visiting the doctor, **citing a range of concerns that were categorized into themes related to healthcare access challenges and medical mistrust, as detailed in the table below**. An additional 20.8% of respondents expressed neutral attitudes, with responses such as "I don't have a problem with it" or "it's alright," suggesting indifference or detachment from the experience.

Themes	Total (23)	Representative Quotes
Feelings of judgment	30.4% (7)	“Don't judge me, I'm being judged. Go meet my family. Why are you judging? Everybody out here has a background. I'm a doctor. I'm the President. So what? I'm a human, you're a human. When I sit down at the doctor I don't want medicine. I want you to understand my family. My life. Understand me. My mental health.” “I just can't deal with doctors who judge me when I tell them something is wrong. If I'm in pain, then believe me.”
Feelings of anxiety and nervousness	26.1% (6)	“It's terrifying, I'm not sure why.” “I feel anxious and nervous. I'm 68 and in good shape now, but life happens. Anything can happen at any given time. Just like you prepare for life you prepare for death.”
Lack of trust in the doctor	21.7% (5)	“It's terrible, I don't trust my current doctor. They're complicit in the system and inconsistent so I study my own health. They give you wrong information.”
Discomfort due to language barrier	13.0% (3)	“La doctora es amable pero no habla español, así que realmente no me entiende.” <i>(English Translation: “My doctor is nice but she does not speak Spanish, so she really does not understand me.”)</i>
Burdened by the given diagnosis	8.7% (2)	“I don't like it when the doctors diagnose me with something, because then I have to deal with it.” “It's bad, it's always something. It's good to stay up to date but I don't like to hear the news. It gives me more stress.”

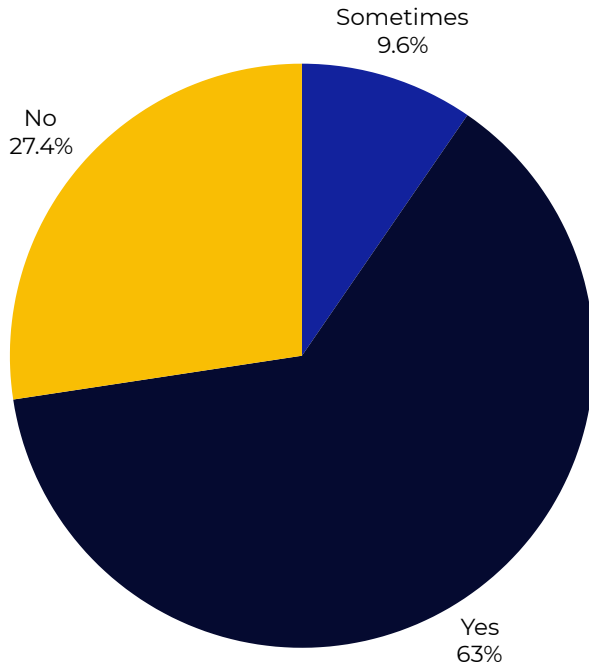
“Do you feel that it is easy or hard to access the doctor? Why?”



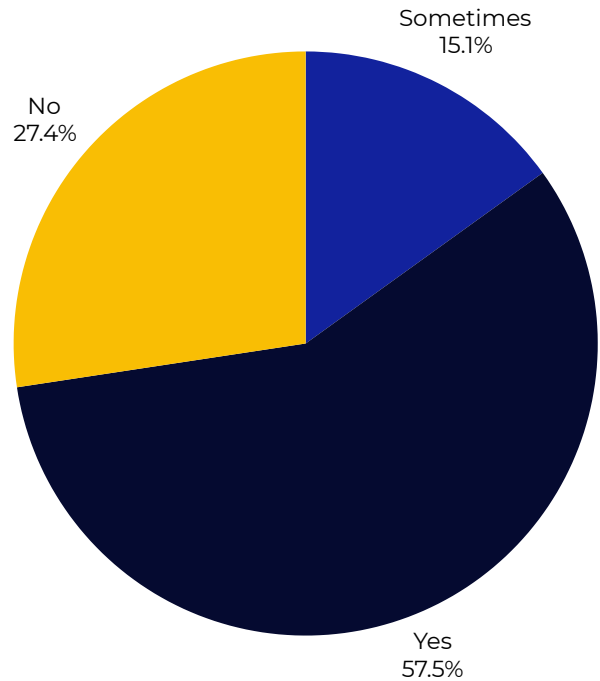
52.0% of respondents to this question (75 respondents) found it challenging to access the doctor, with the most frequently cited barriers being a lack of transportation and challenges with insurance (the latter primarily coming from those who are uninsured).

Themes	Total (38)	Representative Quotes
Difficult due to lack of transportation	28.9% (11)	“It’s hard to get transportation to the doctors' appointments, and it's difficult to get there on time.” “I don't walk good these days so it's hard to get there. If I have a bike it's okay, but I don't want to leave my stuff in the waiting room because someone will take it.” “Hard. Got to walk and my walker got stolen.”
Challenges with insurance	28.9% (11)	“It's hard, you need insurance. If you don't have insurance you can't get help.” “It is hard because insurance makes it difficult. There’s so many different plans and when you go, you have to wait. They didn't see me last time I went for a broken tooth.”
Long wait times to receive an appointment	23.7% (9)	“It takes all day to see a doctor.” “It's hard. You have to wait so long you have a problem and you can't even see them.”
Lack of support to navigate services and paperwork	10.5% (4)	“It's hard to know where to go around here. I need some direction.” “Honestly just filling out the paperwork is a challenge. I can't read or write well.”
Difficult communicating with the clinic without a cell phone	7.9% (3)	“My cell phone gets stolen all the time so I cannot call for an appointment.” “If you have the right communication its easy. But my phone gets stolen all the time.”

Do you feel like the doctor understands you?

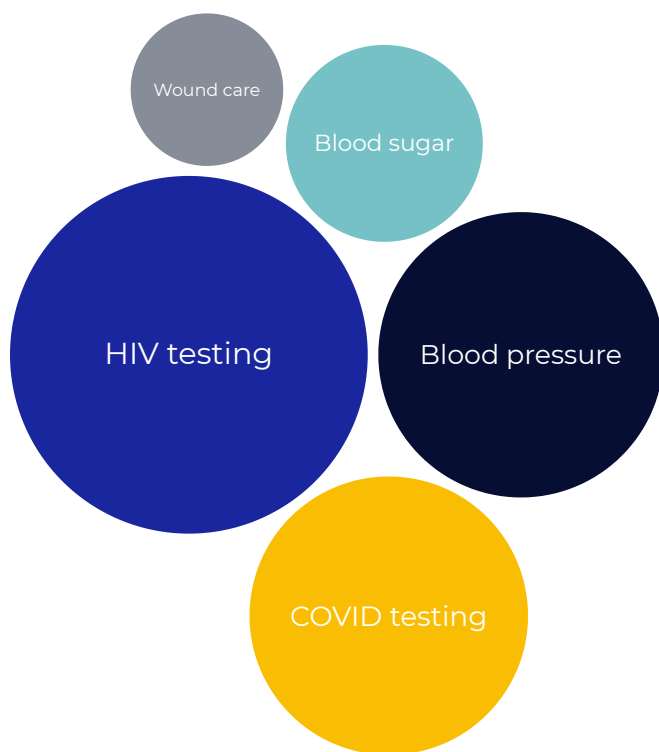


Do you feel like you can trust the doctor?



While most respondents (73) reported feeling like the doctor understood them and that they could trust the doctor, **27.4% cited sentiments of medical mistrust, in addition to 15.1% that said they can trust the doctor only “sometimes.”** Responses related to medical mistrust were categorized into themes, as detailed in the table below.

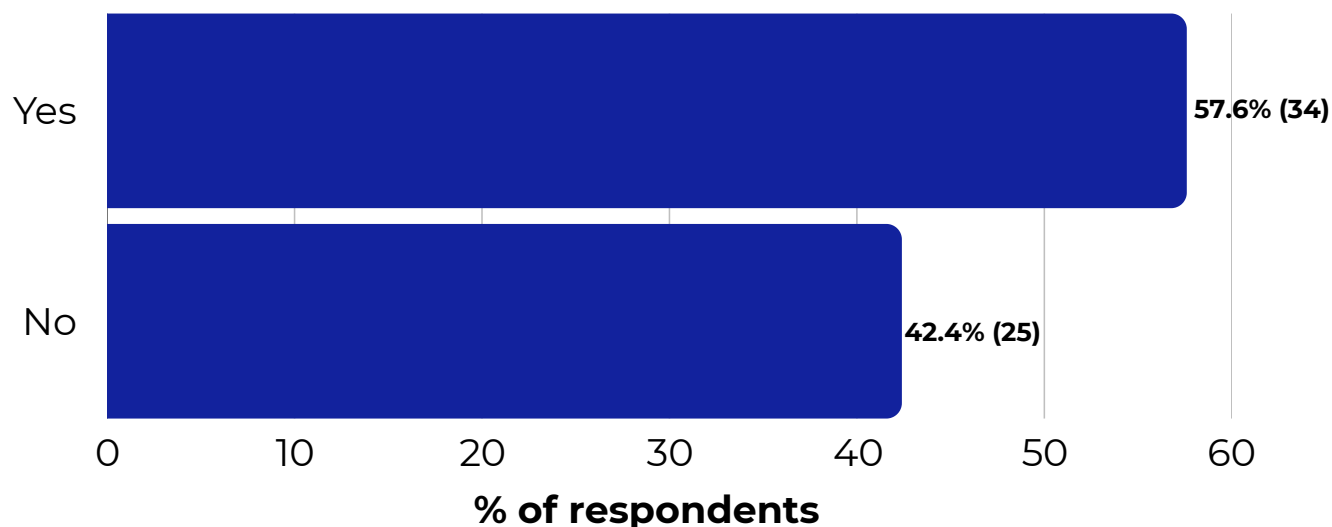
Themes	Total (25)	Representative Quotes
Dismissal of symptoms and perceived stereotyping and biases	48.0% (12)	“If they see that I’m homeless then they treat me different. If I tell them I’m on drugs then they also treat me with no respect.” “Soy moreno. Me ven y me tratan sin respeto. Como que no soy humano.” <i>(English translation: I’m dark-skinned. They see me and treat me without respect. Like I’m not human.)</i>
Poor communication	32.0% (8)	“He [The doctor] doesn’t explain things well. I leave confused.” “My primary doctors don’t listen. I have to tell them what is wrong too many times.”
New doctor at every visit	12.0% (3)	“At the free clinics around here you can see a doctor, but it’s always a new one. So I feel like they don’t really know me.”
Language barriers	8.0% (2)	“No puedo entenderlos.” <i>(English translation: I cannot understand them.)</i>



“If you have used mobile health services what health screenings, testing, or treatment have you received?”

HIV testing: 55.9% (33)
 Blood pressure check: 35.6% (21)
 COVID testing: 33.9% (20)
 Blood sugar check: 16.9% (10)
 Vaccinations: 16.9% (10)
 Medications: 15.3% (9)
 Other STD testing: 13.6% (8)
 Physical exam: 11.9% (7)
 Tuberculosis testing: 11.9% (7)
 Dental care: 8.5% (5)
 Wound care: 10.2% (6)
 Mental health screening: 10.2% (6)
 Colon/Prostate exam: 5.1% (3)
 Hepatitis screening: 5.1% (3)

“Have you received this testing or treatment in a hospital, clinic, or doctor's office as well?”

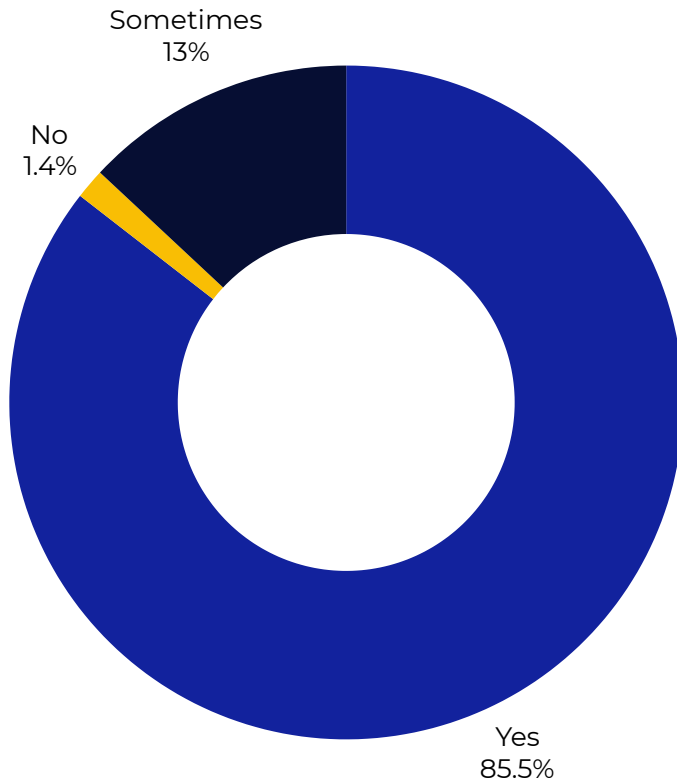


42.4% of individuals who had previously received health services via street medicine or mobile health teams **had only ever accessed these services on the streets**, rather than in a traditional doctor's office.

For these individuals in Skid Row, street medicine and mobile health services often serve as their primary, and sometimes only, access to healthcare, highlighting the importance of investing further into these programs.

“Do you feel like street medicine and mobile health services are helpful?”

Why or why not?”



85.5% (of 75 respondents) expressed that street medicine and mobile health services were helpful. An additional 13.0% stated that they were sometimes helpful. This overwhelming approval speaks volumes about the necessary role that these services play in Skid Row. Key themes that emerged from respondents included:

- **Convenience:** The ability to receive care without the barriers typically associated with traditional healthcare settings, such as long wait times or complex appointment processes.
- **Accessibility:** Mobile health teams bring services directly to individuals where they live, reducing the logistical hurdles that can prevent people from seeking help, including physical disabilities.
- **Referrals to Other Resources:** Street medicine teams often provide not just medical care but also connections to housing, legal assistance, and other support services, addressing the holistic needs of individuals experiencing homelessness.
- **Respectful Interactions:** Respondents noted a distinct sense of respect and friendliness from healthcare workers involved in street medicine and mobile health, fostering a supportive environment that encourages individuals to seek care.

Quotes from respondents:

“If it were not for them many would be dead on the streets.”

“They’re helpful because a lot of people cannot get to the doctor due to lack of appointments or disabilities.”

“It’s a reminder to take care of your health. Your services are convenient, and give peace of mind especially for blood pressure checks.”

“They offer resources that I am unaware of.”

“They are respectful. Usually they don't dismiss. Would love to see them more.”

“Y'all are giving back and making sure we're taken care of. It can be more than just a health problem. It can also be drugs. You understand people's lives and living situation.”

“It helps a lot of people who aren't willing to go to doctors. You guys are the ones who listen, you're a lot of help. I don't check my blood pressure I'm too scared to do it even though it's dangerously high. I only let you guys check it.”

However...

"I've never received a follow-up.
I was supposed to."

"I'd prefer if they could check in on me more often. Like come to my door. I can't walk too good so I can't really get up and go to the van [mobile health unit]."

"Well you guys can't do surgery out here. When it gets to that point then we still are kinda on our own."

"I think some people out here are still afraid to use them. If the services aren't right in front of you then it's still hard to have the hope and motivation to use them. I know because I've been out here. I'd be willing to be that person to bridge the gap. Since I've been out here and people know me, it would make a bigger difference coming from me. I'd be involved in any way I can."

"They need to be out here posted up on the streets. Like you have the Community Refresh Spot open 24/7, but have these guys all over the place all the time so they know where it's at. Just like people know where they can get a cup of coffee. But have fellow people like us welcoming people in. Otherwise it's intimidating, you know? I'd be that [welcoming] person in a heartbeat though."

"They're helpful, but the wait times can still be long. You have to get there before they do. If I see a long line, then forget it. It's not like you get a follow up after that either so for my issues it's not as helpful. For others I'm sure it is."

21.3%

21.3% of respondents who responded to "Do you feel like street medicine and mobile health services are helpful? Why or why not?" (75 respondents) **said that they have experienced issues with continuity of care with street medicine/mobile health teams.** In other words, they do not feel that they are receiving adequate follow-up and believe these services would be more helpful if they provided more continuous, longitudinal care.

48.0%

48.0% of those who reported medical mistrust (25) reported feelings of stereotyping, discrimination, and biases from medical providers. It's evident that these sentiments surrounding healthcare services persist in the Skid Row community.

CALL TO ACTION



Message to LA officials, policy makers, and healthcare workers:

Street medicine and mobile health services play a clear role in improving healthcare within the Skid Row community. About 98% of respondents found these services helpful or somewhat helpful, reinforcing their importance, especially as 52.0% of respondents reported difficulties accessing healthcare, primarily due to transportation and insurance barriers. Notably, 42.4% of individuals who had received services via street medicine or mobile health teams had only ever accessed those specific services through these programs, particularly HIV testing.

But, progress still needs to be made.

It's clear that **significant gaps remain** in meeting basic health needs even with current street medicine/mobile health models in place, as 71.2% of respondents requested food assistance, housing assistance (65.8%), HIV testing + counseling (47.9%), primary care (45.2%), mental health care (45.2%), COVID-19 testing (45.2%), and education services (42.5%).

Further, 21.3% of respondents shared that they felt street medicine/mobile health teams would be more helpful if they provided more consistent follow-up care. 48.0% of those who reported medical mistrust stated that these sentiments came from perceived discrimination and stereotyping by medical professionals.

Interestingly, some community members in Skid Row have suggested **involving individuals with lived experience in Skid Row (even volunteering themselves)** to address feelings of intimidation surrounding the use of street medicine/mobile health services that they feel are present in the community due to underlying medical mistrust. They could also certainly be involved to fill gaps in continuity of care.

This aligns with the **Community Health Worker (CHW) model**, which has proven effective worldwide. In these contexts, we've seen CHWs take a more involved role in providing daily patient visits, escorting people to care, providing food assistance services, arranging transportation, and providing housing assistance as well as emotional support via group therapy sessions. With these systems in place, Partners in Health has reported remarkable results. For example, in Haiti, they reported a 100% clinical cure in all patients receiving CHW support in addition to free care, versus only 56% cure and 10% mortality in patients receiving free care alone, among many other cases worldwide which can be found here:

DOI: [10.1016/S2214-109X\(18\)30073-1](https://doi.org/10.1016/S2214-109X(18)30073-1).

Amplifying and unifying such programs in Skid Row not just as a supplementary intervention—but as foundational to a comprehensive healthcare system—could be the **next step forward**. While street medicine/mobile health teams have filled a critical need with overwhelming community support, challenges such as lack of follow-ups and long wait times often stem from limited capacity and high demand on street medicine/mobile health teams themselves.

By expanding street medicine/mobile health efforts through a robust, unified CHW initiative—led by those with lived experience in Skid Row—we can build a more **sustainable and equitable healthcare system** for this community.

We are eager to engage in further discussion and encourage you to reach out to Erica Robinson (contact@healthmatters.clinic) and Sara Habibipour (sarahabibipour@g.ucla.edu).

ACKNOWLEDGEMENTS

We would like to extend our deepest gratitude to the Health Matters Clinic Helping and Healing Team for their diligent efforts in conducting these surveys and interviewing individuals in Skid Row, which have provided invaluable insights for this report.

Special thanks to the Health Matters Clinic Education and Training Committee for their thoughtful work in developing the survey questions that guided our research.

Lastly, we would like to acknowledge Erica Robinson, President of Health Matters Clinic, for providing the time and resources for this initiative. We would like to acknowledge Sara Habibipour for analyzing the surveys, writing, and creating the report.

